

Transcript

HearO discovery : Thinkitive - 2026/08/13 12:30 IST – Transcript

Attendees

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Transcript

Stavit Cohen: to start with the back office Anita because I had a follow-up discussion with so I can clarify things okay...

Anita Kankate: Okay. Okay.

Stavit Cohen: if it's possible okay so regarding the back office we have initial I would say design what we need to have in this design is to have more accessibility I would say more insight demanding that there is a lot of data but it's not clear what is the outcome from this data. Okay, so I want you to get this initial design the exact she know that what needs to improve and she will summarize that. So this is the main thing.

Anita Kankate: Enter.

Stavit Cohen: What is the name of the external tool that we actually u investigated for support intercom?

איילת ארליכמן: Intercom.

Stavit Cohen: So there is a off-the-shelf tool intercom that allows a tier three support and also a ability to communicate with the clinic or with the customers. So we thought about something like that that can be I don't know integrated maybe you have it.

Stavit Cohen: So this is something that you need to consider as well for support but cleaning what's Yeah. No no no it's just an example.

Anita Kankate: So first want to understand so intercom we need to indicate right in our system.

Anita Kankate: So this will be called your team can support to the patient for if they have any issue right and...

Stavit Cohen: Yeah. We need So,...

Anita Kankate: from patient side they can raise the

Anita Kankate: It is.

איילת ארליכמן: It's technical falls for the back office to see if we have a issues with the system with the uploading with the recording something like that.

Stavit Cohen: so I think the purpose one is for the technical support to understand if there is an issue and also to the patient needs to track that's correct there should be a ticket that we should track. you are correct and there should be normally the clinic will communicate with the tech support.

Pooja Mutadak: Okay.

Stavit Cohen: It's either the clinic or the patient. So both needs to have a separate ticket.

Anita Kankate: But that patient is above 65.

Stavit Cohen: That they need to track

Anita Kankate: So they can raise the ticket via text we can say text or they can add wise command.

Anita Kankate: So I have this issue and we can receive that ticket. So, do you have anything in mind And so from patient side how they can raise the ticket and...

Pooja Mutadak: I think you have a interface raise a ticket in a mobile and...

Pooja Mutadak: both on a clinical side as well a support icon where they can give a support details what exactly the issue the incident management we will say and inside the back office we will have that incidences. So with the priorities like that we can do easily. Yeah. Yes.

Anita Kankate: also we can define the predefined what is the issue

Anita Kankate: they are getting. So they have to only select and they can send to the back office. We can add Yeah.

Pooja Mutadak: I think we can come up with that support plan. but I get it. Basically this support is more for if anything related to the software that support ticket should be visible in a back office so that we as a team can take action on them.

Stavit Cohen: Excellent. No.

Anita Kankate: And in that ticket we can show the patient ID as well as organization.

Anita Kankate: So it will be defined from where I have already that right.

Pooja Mutadak: Yes, I think Anita we can just come up with that flow...

Pooja Mutadak: where we can have that screen design but this is clear. Yeah.

Anita Kankate: Okay. Yes.

Stavit Cohen: So this is one aspect.

Stavit Cohen: The other aspect that there are some redundant fields in the back office. I don't want you to deal with that in so you are revising this page. If not you don't need to do a cleanup of all the back office. It's not a we will postpone that to the next version. Only if you touch this page then if you have a redundant fields stuff you can remove it but if it's not part of the scope of work ...

Pooja Mutadak: Yep.

Stavit Cohen: if you are not touching this page you don't need to clean everything old things you don't need to deal with that okay as long as you know it makes sense okay you don't need to do a re full rearchitecture because we have a lot of things that related to the voice engine that we are currently

Stavit Cohen: utilizing in the back office and we need to maintain that. So, another thing that I would like to mention that we need to consider a I would say level of support. what does it mean? We have the clinics. And we need to have something that because there are many patients. We need to have a overall dashboard that highlights what are the critical issues that the support needs to deal with and what are the alerts and how alert is defined and how you notify this alert you send an email what is the mechanism to escalate so I think it's something that also needs to be considered because currently it's not the only thing that the clinic gets is a weekly email

00:05:00

Stavit Cohen: And there is no escalation in term of system health instead of compliance issues. It's weekly I get it but there is no escalation. So we need to think about escalation mechanism and alerts mechanism. It's not yet embedded in the support system. So we need to think what is the minimum that is required from how a support has a very good overview of what are the issues.

Stavit Cohen: I call it accessibility because the information need to be accessible and understood and not a lot of information because if there is a lot of information people will miss it.

Stavit Cohen: So we need to think about of the messages. It's very important and...

Pooja Mutadak: Yeah, that.

Stavit Cohen: we talked about it with but you need to take that into account.

Pooja Mutadak: Yep. Okay.

Stavit Cohen: So from my standpoint, those are the guidelines from that perspective. Take it. You can leverage a yell to whatever you need and she will support you if you have question but you can take it from here. Okay.

Anita Kankate: stuff.

Anita Kankate: For back offices, how many roles do you have currently already practice admin, support team. Okay.

Stavit Cohen: I please answer because I know the exact I think that the supervisor is the support. No.

איילת ארליכמן: Yeah, we don't. There are few roles. It's a bit of a mess, I guess. I need to check it, but we don't have any support right now. We have a regular and supervisor and something like that. But we need to fix this to can do everything and...

איילת ארליכמן: can see all the screens and we need to define it better. I guess that's the role.

Ilan Shallom: Not we puja and...

Ilan Shallom: our team we define it. There are tears. The first tier is the cleric itself where it can handle things. Then there's the third tier and the fourth is our team that will come and solve the problem. So you define the tears and give it to us.

Anita Kankate: Elan this is for cardio side.

Anita Kankate: I am asking cardio already developed the back office. There will be cardio admin one portal support team for another portal and do you want to any other role in that clinical supervisor

Ilan Shallom: Wait to We can't hear you.

Stavit Cohen: I suggest that there will be a support administrator.

Stavit Cohen: And I think also on the support we need to define at least two level of support.

Stavit Cohen: because we don't want so...

Ilan Shallom: We lost communicate. No, we can't hear you.

Stavit Cohen: what I'm saying yes can you hear me okay I want to repeat so ...

Anita Kankate: Yeah, it's now.

Ilan Shallom: Yes. Then you are not clear.

Pooja Mutadak: a little bit lost to hear you. Yeah.

Anita Kankate: Can you please repeat again that sentence? So I let do you know...

Stavit Cohen: I think we need to have a two level support.

Ilan Shallom: We understood that you requested two layers but there we stop hearing you.

Anita Kankate: how many portals you have developed for backup pieces?

איילת ארליכמן: How many?

Anita Kankate: portals like roles we have created as of now.

00:10:00

איילת ארליכמן: I can check and tell you exactly...

Anita Kankate: Got it. Yes.

איילת ארליכמן: but what is I think is not the best practice and we should make it better as it said.

Ilan Shallom: I think we need your assistance here...

Ilan Shallom: but the guideline will hear from stud but you define it. We want I'll ask to off the call.

Ilan Shallom: I'll ask to define it if she'll return. So, you can continue Anita Puja with your questions.

Anita Kankate: Yeah and...

Anita Kankate: also let me share my screen.

Ilan Shallom: Yes.

Anita Kankate: Yeah are you able to see my screen? Yeah. So this type of dashboard we can add for back offices. As of now we have added per clinic how many patient are there. Here we can see how many minutes they have logged. so here we can see the all.

Ilan Shallom: Yeah, didn't join yet.

Anita Kankate: Yeah, I have admitted to yeah.

Ilan Shallom: I see. But I see my

Anita Kankate: So this type of dashboard we can add here.

Anita Kankate: Here we can see how many totals count enrolled patient provider user or patients and this is the device recording how many records we have received day so we can configure dashboard we also know about what type of statistic you require here and as you mentioned you need to create the new organization. So in this we are asking adding provider group...

Anita Kankate: but in your case it will be We can collect organization details their address website what is the billing address for that and onboard that practice.

Ilan Shallom: You can go back to the previous So this is the overall view and...

Anita Kankate: Yes, you can see.

Ilan Shallom: I can deep dive to certain HMO and see the view for a certain HMO.

Anita Kankate: So here you can see the multiple organization. If you select that you can see specific organization details as well.

Ilan Shallom: And when I am okay that's great and when I want to deep deeper and to see a single patient picture is there I want to see a certain patient in Athena.

Ilan Shallom: So will I be able to deep dive and see it information or dashboard of the patient?

Anita Kankate: So here we can see the multiple list of organization already we have registered.

Ilan Shallom: Okay.

Anita Kankate: If you can open that you can see the organization profile how many users or care team we have enrolled. We can see that details and this is the patient details we have added.

Ilan Shallom: And we can deep dive to see is the return to the first ST.

Pooja Mutadak: Yeah so I think what he's asking for if you go to the analytics screen can you go to the analytics so here he is interested in seeing the details for per patient ...

Ilan Shallom: Yeah. in single side put I would like to see the full picture to deep dive to HMO and...

Anita Kankate: that we can stop. person.

Pooja Mutadak: how we can see the recording their device information their encounter Yep. the provides. yes.

Ilan Shallom: to deep dive and see a single patient dashboard.

Pooja Mutadak: Okay. Yeah.

Ilan Shallom:

Ilan Shallom: This is something practical. If I see I can very fast deep dive and see the details of the problem. maybe it will be clear from the first screen but maybe when I deep dive to certain HMO I see the picture there.

00:15:00

Pooja Mutadak: Yep. Okay.

Ilan Shallom: But even if I'm the HMO, I would like to be able to see a single patient.

Anita Kankate: Yeah.

אילית ארליכמן: We already did some wireframe like that. So I can send you you can I think more understand what we think I meant You see my screen in the dashboard?

Ilan Shallom: I didn't understand What did you just said? Yeah.

Ilan Shallom: Yes. Yes.

אילית ארליכמן: So what we did already is this is the level of the overview if we have some issues open it's categorized by compliance voice engine sensors it's only a thought that we had only a wire frame okay...

Anita Kankate: I just want to understand here.

אילית ארליכמן:

אילית ארליכמן: but it's an idea here...

Anita Kankate: Do you have added any which organization we have received this issue? Yes.

אילית ארליכמן: if you No, I don't remember what I did. Okay, maybe I did. It's an issue that the system has.

Pooja Mutadak: Got it.

אילית ארליכמן: Okay, in this overview, it's a system issues. Okay, and I have also a system health. So inside here.

Stavit Cohen: Anita is right.

Stavit Cohen: Anita is right. We need to doization.

אילית ארליכמן: Okay.

אילית ארליכמן: You can take it from So here is another dashboard the system health. So here you do have a organizations you can check and...

Stavit Cohen: Which one?

אילית ארליכמן: it is a component with different topics that can highlight also issues that we have.

אילית ארליכמן: I added here all kinds of issues that ego reported. So it should be here.

אילית ארליכמן: So we can see if he has some problem and also another dashboard related to the clinic. So I will provide it.

Anita Kankate: Okay.

אילית ארליכמן: You can take it from here. And

Stavit Cohen: But there is something that I want to kind of iterate.

Pooja Mutadak: Yeah.

Stavit Cohen: Environment for the EU Europe environment for example. So it's a multi architecture. So I can't use

Ilan Shallom: Stop it. We can't hear you once again. It's the clicky voice. No, we can't hear use of Bad quality. We can't hear use of it.

Ilan Shallom: I don't know how to share. I wanted to show you something also. how I do share here?

Pooja Mutadak: There is a share icon next to the video.

Ilan Shallom: Okay.

Pooja Mutadak: So first is the audio next to the video and next to yeah so once you share it will give you the option do we have to share a specific window or...

Ilan Shallom: Share and how I see my windows here. let me h just a moment.

Pooja Mutadak: a complete screen share just hit that yeah okay not

Ilan Shallom: Okay, that's nice. I'm doing share. You see this one? Okay. So, this is my thought about screens. So, I have here the total screen. It's not nice information but is the idea.

Ilan Shallom: by center I can see a center and here I go to a center and see the information inside the center details information and I don't recall but I also had a way of seeing it by last four months or what last year.

00:20:00

Ilan Shallom: So the analyze of the top screen anita can you return to the first screen that if I can see the history about what I'm doing the statistic if this is only the last week or last month or last half here. so this ability would enable the me to locate...

Ilan Shallom: where it happened because if the statistic is bad only on the last week I understand that we have a problem from this day to that day seven days and hey

Pooja Mutadak: Okay. ...

Pooja Mutadak: if I summarize this correctly on the dashboard side so we are more interested in a four different kind of dashboard one is like as the cardio we should able to see a complete view of all the clinics and all another will be giving us the per clinic details and the third would be more specific to the patient and all of these data we need to have with the time range whether for last

Pooja Mutadak: 7 days per day, 24 hours. He Okay.

Ilan Shallom: Exactly.

Ilan Shallom: To be able to fast locate the root cause of the problem.

Pooja Mutadak: Yeah. Got So for the patient we are more interested to see the readings more of 1 hour last 24/7 like that we will see organizational will be one view then for all organization there would be separate view and there more is a support that support will give us the information about the

support tickets. So that kind of four dashboard we are more interested to give us the complete overview.

Pooja Mutadak: Yeah. Yes.

Ilan Shallom: This is the idea.

Ilan Shallom: You are the boss. You said exction.

Pooja Mutadak: Yes. Yes. We are just gathering the ideas so we can come up with the screen design for it. But I just want to summarize it so the team will also understand what kind of design we have to get it. Yeah. Okay.

Ilan Shallom: She will rejoin us.

Pooja Mutadak: Yeah, I think we can move further then.

Anita Kankate: So, are we waiting for Yeah.

Ilan Shallom:

Ilan Shallom: She lost connection. I don't think I just let me see just a moment.

Anita Kankate: And also I want to show...

Ilan Shallom: The problem she can't use the teams.

Anita Kankate: what are Yeah, we have scheduled a call on Google meet now. Yep.

Anita Kankate: Elon, are you talking with us or Yes.

Ilan Shallom: you hear me? okay. So my question was can you switch to teams or you need to use the Google meet because the street cannot use the Google meet now on the go. So

00:25:00

Anita Kankate: Maybe I let can you share here link of teams?

אילית ארליכמן: Yeah. Heat.

Anita Kankate: Yeah, you can add here over the chat so we can join that call.

Ilan Shallom: You want me to send the invitation to teams?

אילית ארליכמן: I'm doing it.

Ilan Shallom: I can.

אילית ארליכמן: I'm doing it right now.

Ilan Shallom: You have done it. I

אילית ארליכמן: I'm doing it. Yeah. And we'll share the link.

Ilan Shallom: Okay, you send this only to us on I share the link.

אילית ארליכמן: I will put it in just a minute. Okay.

Ilan Shallom: To leave this meeting. Okay. Bye. See you in a couple of minutes.

Anita Kankate: Yes. Yeah.

transcript notes

Yes. I went through the transcript and separated **what was actually discussed** from the **concrete design-team action items**.

1. What the discussion was mainly about

The meeting was primarily about **reworking the HearO Back Office experience**, especially making the large amount of operational data more understandable and actionable.

The main concern from the client side was:

There is a lot of data in the current Back Office, but it isn't clear enough what the important outcome/action is.

They want the design to focus on **accessibility of information, prioritization, system health, alerts, and fast drill-down into problems**, rather than simply showing more data.

There were essentially **4 major areas** discussed:

1. **Support / Incident Management**
 2. **Back Office Dashboard / System Health**
 3. **Drill-down from overall → organization/clinic → patient**
 4. **Alerts, escalation and time-based analytics**
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2. Design team action items

A. Redesign the Back Office dashboard

The existing dashboard needs to evolve from being primarily a data/statistics display into something that helps the support team **quickly identify problems and understand what needs attention**.

Design needs to explore:

- Overall system health
- Critical issues
- Alerts
- Issues requiring action
- Severity/prioritization
- Organizations/clinics affected
- Clear messaging rather than information overload

The client specifically emphasized that information should be **accessible and understandable**, because too much information can cause important issues to be missed.

Action: Rework the dashboard hierarchy so the first screen answers:

"What is wrong right now, how serious is it, and where is it happening?"

B. Design the 4-dashboard structure

This was probably the clearest design direction from the meeting.

The team discussed **four different dashboard/views**:

1. Overall / All Organizations Dashboard

High-level view across all clinics/organizations.

Should help the support/admin team understand:

- Overall system status
- Which organizations have issues
- Critical issues
- System health
- Overall trends/statistics

2. Organization / Clinic Dashboard

When selecting a particular organization/clinic, users should be able to see its specific situation.

For example:

- Organization health
- Issues
- Compliance
- Voice engine
- Sensors
- Patient/provider information
- Relevant operational statistics

3. Patient Dashboard

Users should be able to **deep dive all the way to an individual patient**.

The discussion specifically asked for the ability to see the patient's complete picture, including things such as:

- Recordings
- Device information
- Encounters
- Readings / relevant patient data

- Problems affecting that patient

The purpose is to make troubleshooting much faster.

4. Support Dashboard

A dedicated area for **support tickets/incidents**.

This would allow the support team to see:

- Open tickets
- Issues
- Priority
- Organization/patient associated with the ticket
- Status
- What needs action

The four-dashboard concept was explicitly summarized during the meeting.

3. Design the support / ticketing flow

This is another clear action item.

The team discussed something similar to **Intercom**, but Intercom was only given as an example/reference, not as a confirmed implementation.

The intended flow:

Patient or Clinic → Raise technical issue → Support ticket → Back Office → Support team investigates → Resolution

Issues could relate to things like:

- Recording problems
- Uploading problems
- Device/system problems
- Other software-related issues

Both **clinic and patient** may raise support issues, and these should be tracked separately as tickets.

Design needs to define:

Patient/Clinic side

- Support entry point

- "Raise a ticket"
- Issue category/predefined issue
- Description/details
- Possibly text/voice input was discussed for patients
- Submit ticket

Back Office

- Support ticket list
- Ticket details
- Priority
- Status
- Patient ID
- Organization/clinic
- Issue type
- Resolution/action

The team specifically suggested predefined issue categories so users don't always have to describe the problem from scratch.

4. Design incident/support prioritization

The Back Office should not just show a list of incidents.

The support team needs to understand:

What is critical? → What can wait? → What needs escalation?

The discussion specifically called out:

- Critical issues
- Alerts
- How alerts are defined
- How users are notified
- Email notification
- Escalation mechanism
- Support overview

Currently, the system mainly provides a weekly email, and there isn't enough system-health escalation.

Design action

Define UI concepts for:

Alert → Severity → Owner → Notification → Escalation → Resolution

You don't necessarily need to design the entire backend logic yet. The important thing is to establish the **UX model**.

5. Design the drill-down hierarchy

This is important.

The intended navigation appears to be:

Overall Back Office



Organization / Clinic



Patient



Patient/system details

The user should be able to move from a high-level problem directly into its source.

For example:

- 3 clinics have recording issues

- Open Clinic A

- See affected patients

- Open Patient X

- See device + recording + encounter information

This "fast root-cause identification" was explicitly the goal of the discussion.

6. Add time-range / historical analysis

This is another important requirement that came up.

The dashboard shouldn't only show **current statistics**.

Users should be able to understand **when a problem started**.

The example discussed was essentially:

- If the statistic is bad only during the last week, I should be able to identify that specific period and understand when the problem occurred.

Design action

Consider time filters such as:

- Last 24 hours
- Last 7 days
- Last 30 days
- Last 4 months
- Last year

The exact final ranges weren't formally finalized, so treat these as **examples from the discussion**, not confirmed requirements.

For patient-level data, the discussion suggested more granular periods such as **1 hour / 24 hours / 7 days**.

7. Review existing Back Office screens — but DON'T redesign everything

This is an important scope clarification.

The client explicitly said:

Do not clean up/rearchitect the entire Back Office right now.

There are existing/redundant fields and older elements, but those can be handled in a later version.

If the design team is already touching a screen and notices redundant fields, they can remove them. But **don't expand the project into a full Back Office redesign**.

So:

Don't do

"Let's redesign the entire Back Office."

Do

"Let's improve the specific screens/flows in scope and clean up obvious redundancy only where we're already working."

8. Review roles and permissions

There was an unresolved discussion around Back Office roles.

They mentioned:

- Admin
- Support
- Supervisor
- Different support tiers

But the client said the current role structure is somewhat messy and needs to be better defined.

They also discussed having **at least two levels of support**, with the broader team potentially defining the tiers.

Design action

Don't invent the final permissions yet.

Instead, keep a **role/permission dependency** open:

Need client confirmation of Back Office roles, support tiers and which screens each role can access.

This is something the design team should flag rather than assume.

9. Use the existing wireframes as input

Ayleth mentioned that they already have some wireframes/concepts around:

- Overview
- Open issues
- Compliance
- Voice engine
- Sensors
- System health
- Organization-related issues

These are **existing ideas/wireframes**, not necessarily final designs. She said she would provide them to the team.

Design requirements

HearO Back Office — UX/UI Design Requirements & Design Execution Document

1. Document Purpose

This document translates the HearO Back Office discovery discussion into a clear UX/UI design brief. Its purpose is to define:

- What the Back Office experience is expected to achieve.
- The information architecture that should guide the redesign.
- The screens and flows the design team needs to create or improve.
- The functional requirements that the UI must support.
- What is intentionally outside the current scope.
- Which items remain dependent on client clarification.

The document is based on the discovery call transcript and the accompanying meeting notes. It does not introduce new product requirements beyond what was discussed.

2. Background

The current HearO Back Office contains a significant amount of operational and system data. The primary concern raised during the discovery discussion was not the lack of data, but the lack of clarity around that data.

Users currently have access to numerous statistics and operational details, but the interface does not make it immediately obvious:

- What is currently wrong.
- Which issue is most important.
- Which organization or patient is affected.
- Whether the issue requires immediate action.
- How to quickly investigate the source of the problem.

The redesign should therefore focus on improving the accessibility, hierarchy, and actionability of information rather than simply adding more metrics or charts.

The Back Office should evolve into a support and monitoring workspace that enables users to identify issues quickly and move efficiently from a high-level overview to the detailed information required to investigate and resolve them.

3. Primary Design Objective

The primary design objective is to create a Back Office experience that answers the following questions as quickly as possible:

1. **What is wrong?**
2. **How serious is it?**
3. **Where is it happening?**
4. **Who or what is affected?**
5. **What should the support user do next?**

The interface should prioritize important information over secondary information and reduce the cognitive effort required to locate operational problems.

Core UX principle

Signal → Priority → Context → Drill-down → Action

The design should avoid presenting large volumes of equally weighted information, as this can make critical issues difficult to notice.

4. Design Goals

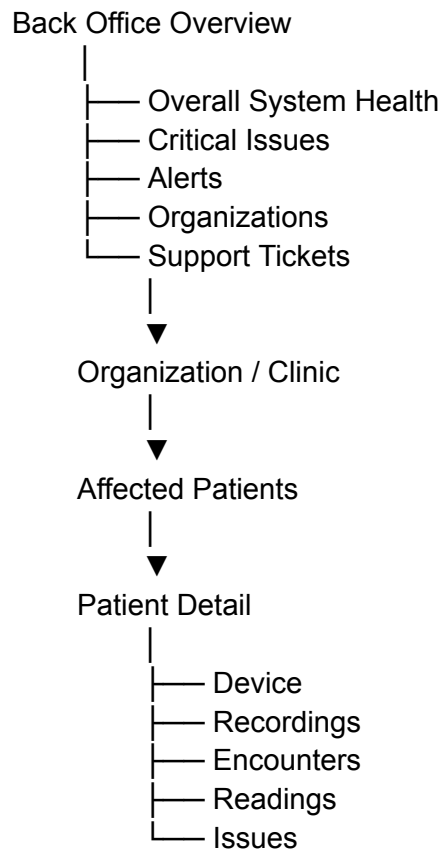
The redesigned Back Office should provide:

- A clear overview of overall system health.
 - Immediate visibility of critical issues and alerts.
 - Clear prioritization of incidents based on severity.
 - Visibility into which organizations or clinics are affected.
 - The ability to drill down into organization-level details.
 - The ability to drill down further into individual patient information.
 - A dedicated support ticket management experience.
 - Historical context through time-based analytics.
 - A foundation for future alerting and escalation workflows.
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5. Information Architecture

The discussion established a layered navigation model.

Primary hierarchy



This hierarchy is intended to support rapid root-cause identification.

A support user should be able to begin with an overall issue and progressively narrow the investigation until the affected patient or system component is identified.

6. Required Design Areas

The design work should cover six primary areas:

1. Back Office Overview Dashboard
 2. Organization / Clinic Dashboard
 3. Patient Detail Dashboard
 4. Support Ticket Dashboard
 5. Ticket Creation and Ticket Detail Flow
 6. Alert and Escalation UX Model
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7. Back Office Overview Dashboard

Purpose

The Overview Dashboard should serve as the primary command center for Back Office users.

It should immediately communicate the health of the system and highlight the issues that require attention.

Current-state concern

The current dashboard appears to focus mainly on statistics such as:

- Total patients.
- Providers/users.
- Number of recordings.
- Minutes logged.
- General counts.

These statistics may remain useful, but they should no longer dominate the interface.

Required information hierarchy

Priority 1 — Immediate attention

The top of the dashboard should focus on:

- Critical issues.
- Active alerts.
- Number of organizations affected.
- Number of patients affected.
- Issues requiring action.

Priority 2 — System health

The next level should communicate:

- Overall system health.
- Trends over time.
- Major issue categories.
- Whether conditions are improving or worsening.

Priority 3 — Operational statistics

Secondary information may include:

- Patient counts.
- Provider counts.
- Recording volumes.
- Usage statistics.
- Other operational metrics.

These should support context rather than compete with critical issues.

Suggested dashboard sections

System Health Summary

Should communicate:

- Current health status.
- Number of critical issues.
- Number of warnings.
- Number of organizations impacted.

Critical Issues

A prioritized list showing:

- Issue title.
- Severity.
- Affected organization(s).
- Affected patient count where applicable.
- Current status.
- Time detected.

Issue Categories

Potential categories discussed include:

- Compliance.
- Voice engine.
- Sensors.
- Recording.
- Uploading.
- Device/system issues.

These categories are based on the discussion and existing wireframe concepts and should be validated before final implementation.

Historical Trends

The dashboard should support understanding whether issues are recent or ongoing.

8. Organization / Clinic Dashboard

Purpose

The Organization Dashboard provides a deeper view into a selected clinic or organization.

Users should be able to navigate here directly from the Overview Dashboard.

Required information

The screen should provide a consolidated picture of the selected organization, potentially including:

Organization context

- Organization name.
- Basic organization details.
- Number of patients.
- Number of providers/users.

Organization health

- Overall health status.
- Open issues.
- Critical issues.
- Alerts.

Issue breakdown

Potential issue categories include:

- Compliance.
- Voice engine.
- Sensors.
- Recording.
- Device-related issues.

Affected patients

The user should be able to identify which patients are impacted by the organization's issues.

This list should act as the entry point to the Patient Dashboard.

Interaction requirement

Users should be able to move seamlessly from:

Overview → Organization → Patient

without losing the context of the issue they are investigating.

9. Patient Detail Dashboard

Purpose

The Patient Dashboard is intended to support troubleshooting.

The discussion emphasized that support users need to see the "complete picture" for an individual patient once they have identified a problem.

Required information

The screen should consolidate relevant information such as:

- Patient information.
- Device information.
- Recordings.
- Encounters.
- Readings.
- Current issues affecting the patient.
- Relevant historical information.

The exact fields and data points should be confirmed with the client before finalizing the UI.

Design intent

The Patient Dashboard should answer questions such as:

- Is the patient's device functioning correctly?
- Are recordings being received?
- Is there an encounter associated with the issue?
- When did the problem begin?
- Is the issue isolated to this patient or part of a larger organization-level problem?

The screen should support investigation rather than simply displaying patient demographics.

10. Support Ticket Management

Purpose

The Back Office requires a dedicated support and incident management experience for software-related issues.

Intercom was mentioned only as an example of the type of support workflow being considered. It should not be treated as a confirmed integration requirement.

Ticket sources

Support issues may be raised by:

- Patients.
- Clinics.

Both sources should create trackable tickets within the Back Office.

Types of issues discussed

Examples include:

- Recording failures.
 - Uploading problems.
 - Device issues.
 - Software/system issues.
-

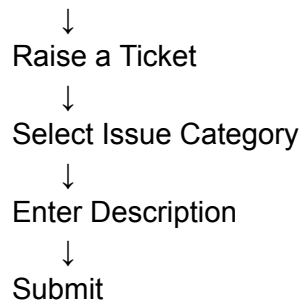
11. Ticket Creation Flow

User-side experience

The support entry point should be simple and easy to understand.

Proposed flow

Help / Support



Required inputs

At minimum, the UI should consider:

- Issue category.
- Description/details.

The discussion also mentioned the possibility of text or voice input for patients. This should be treated as a concept discussed during discovery, not as a finalized requirement.

Predefined issue categories

The design should support predefined categories to reduce the effort required to report an issue.

12. Support Ticket Dashboard

Purpose

The Support Dashboard should allow support users to manage incidents efficiently.

It should not simply display an unprioritized list of tickets.

Required information

The ticket list should support visibility into:

- Ticket ID.
- Issue type.
- Priority.
- Status.
- Organization/clinic.

- Patient ID where applicable.
- Date/time created.
- Assigned owner, if supported.
- Current action required.

Design priority

The support user should be able to quickly determine:

- Which tickets are critical.
 - Which tickets can wait.
 - Which tickets require escalation.
 - Which tickets are awaiting action.
-

13. Ticket Detail Screen

The ticket detail view should provide enough context to investigate and resolve the issue.

Potential sections

Ticket summary

- Ticket ID.
- Issue category.
- Description.
- Priority.
- Status.

Context

- Organization.
- Patient.
- Related system/device information where applicable.

Activity

- Creation date.
- Updates.
- Status changes.
- Resolution information.

Actions

Depending on permissions and future product decisions, actions may include:

- Assign.
- Change priority.
- Update status.
- Escalate.
- Resolve.

Final actions should be confirmed with the product team.

14. Alert and Escalation UX

Problem identified

The current experience reportedly relies mainly on weekly email communication and lacks a strong system-health escalation mechanism.

The new design should establish a clear UX model for alerts.

Required UX model

Issue Detected



Severity Assigned



Owner Identified



Notification Sent



Escalation if Unresolved



Resolution

Design considerations

The interface should make it easy to understand:

- What triggered the alert.
- How severe it is.
- Who owns the issue.
- Whether a notification has been sent.
- Whether the issue has been escalated.

- Whether it has been resolved.

The backend rules for alerts and escalation do not need to be designed at this stage. The immediate goal is to establish the UX model and information hierarchy.

15. Time-Based Analytics

Purpose

Users need to understand not only the current state of the system but also when a problem began.

Historical context is important for identifying root causes.

Design requirement

Dashboards should support time filtering.

Examples discussed include:

- Last 24 hours.
- Last 7 days.
- Last 30 days.
- Last 4 months.
- Last year.

These ranges were discussed as examples and should be validated before finalizing the interface.

For patient-level information, more granular ranges such as:

- 1 hour.
- 24 hours.
- 7 days.

may be appropriate.

UX outcome

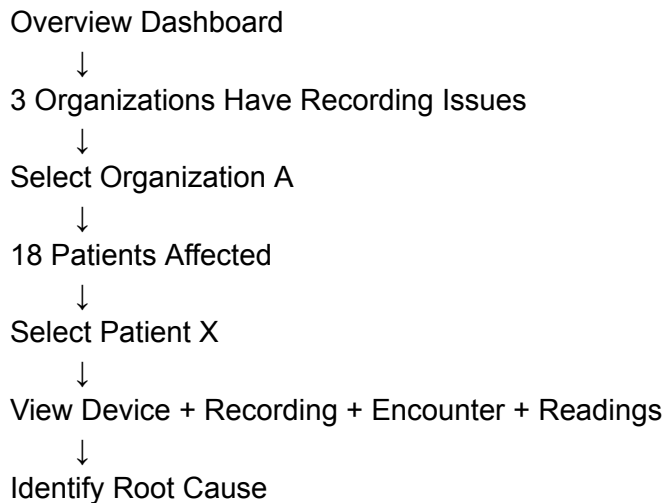
The user should be able to identify patterns such as:

- The issue only began this week.
- The issue has existed for several months.
- The problem occurred during a specific period.

16. Drill-Down Requirements

Drill-down is a central requirement of the design.

Example workflow



Every major dashboard should provide clear entry points to the next level of detail.

The user should not have to search manually for the affected organization or patient after identifying an issue.

17. Existing Wireframes

The client indicated that existing wireframes or concepts already exist for areas such as:

- Overview.
- Open issues.
- Compliance.
- Voice engine.
- Sensors.
- System health.
- Organization-related issues.

These should be reviewed before starting the detailed UI work.

The design team should use them as reference material and evolve them to satisfy the clarified requirements from the discovery discussion.

They should not automatically be treated as final designs.

18. Scope Boundaries

In scope

The current work should focus on:

- Improving the Back Office dashboard hierarchy.
- Designing the organization drill-down.
- Designing the patient drill-down.
- Designing the support ticket workflow.
- Designing support prioritization.
- Establishing the alert/escalation UX model.
- Improving screens that are already being touched as part of this work.

Out of scope

The client explicitly stated that the team should not:

- Redesign the entire Back Office.
- Re-architect every existing screen.
- Clean up all legacy fields.
- Attempt to solve every existing UI inconsistency.

If a screen is already being redesigned and contains obvious redundant fields, those may be removed as part of that work.

The project should remain focused on the agreed support, monitoring, and drill-down experience.

19. Open Dependencies

The following items require client or product confirmation before they should be finalized in the UI.

Topic	Status
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Exact Back Office roles	Open
Role permissions	Open
Support tiers	Open
Ticket ownership rules	Open
Alert definitions	Open
Escalation rules	Open
Final time ranges	Open
Exact patient-level data fields	Open
Final dashboard metrics	Open
Voice input for tickets	Discussed, not confirmed
Intercom integration	Reference only, not confirmed

The design team should clearly flag these items rather than making assumptions.

20. Design Deliverables

The UX/UI team should produce the following deliverables.

1. Back Office Overview Dashboard

Should include:

- System health.
- Critical issues.
- Alerts.
- Affected organizations.
- Time filters.
- Trend information.
- Drill-down entry points.

2. Organization Dashboard

Should include:

- Organization context.
- Health summary.

- Issue breakdown.
- Affected patients.
- Historical trends.

3. Patient Dashboard

Should include:

- Patient context.
- Device information.
- Recordings.
- Encounters.
- Readings.
- Active issues.
- Historical information.

4. Support Dashboard

Should include:

- Ticket queue.
- Priority.
- Status.
- Organization.
- Patient.
- Issue category.
- Owner/assignment where applicable.
- Escalation indicators.

5. Ticket Creation Flow

Should include:

- Support entry point.
- Issue category selection.
- Description.
- Submission confirmation.

6. Ticket Detail Screen

Should include:

- Ticket summary.
- Organization and patient context.
- Activity/history.
- Priority and status.

- Available actions.

7. Alert/Escalation UX

Should establish:

- Severity hierarchy.
 - Notification states.
 - Escalation states.
 - Resolution states.
-

21. Acceptance Criteria from a UX Perspective

The design should be considered successful if a Back Office support user can:

1. Open the dashboard and immediately understand the current health of the system.
 2. Identify the most critical issue without scanning large amounts of data.
 3. See which organization is affected.
 4. Drill into the organization to understand the scope of the problem.
 5. Identify the affected patient or patients.
 6. Open a patient view and inspect the relevant device, recording, encounter, and reading information.
 7. View the related support ticket if one exists.
 8. Understand the issue's priority, status, and whether escalation is required.
 9. Review historical data to determine when the issue began.
-

22. Final Design Direction

The Back Office should not be designed as a collection of dashboards filled with statistics.

It should function as an operational support workspace where information is progressively revealed according to importance and investigation needs.

The experience should guide the user through the following sequence:

Detect the issue → Understand its severity → Identify the affected organization → Drill into the affected patient or system component → Review the relevant context → Take or initiate the appropriate support action.

This principle should guide the visual hierarchy, navigation, interactions, and prioritization across all Back Office screens created as part of this phase of work.